Which specialties, and what they cost

Demand, competition and capital, for the Abuja campus.

One thing has changed the answer. Three of the four specialties that drive Nigeria's medical tourism outflow are exactly what the African Medical Centre of Excellence opened in Abuja with. It is backed by Afreximbank and King's College Hospital and runs 170 beds going to 500, five theatres, three catheterisation laboratories, a 20-bed intensive care unit and the largest stem cell laboratory in the region. **Oncology, cardiology and haematology in Abuja are now taken.** The opportunity for this campus is the ambulatory layer beneath and beside that, which AMCE will not serve and does not want.

01 / WHERE THE DEMAND IS

Four specialties carry most of the outflow.

Signal	Figure	Detail
Outbound medical spend, 2025	USD 685 M	Up from 643 M in 2024. Q3 2025 alone was 208 M, about 18% above the same quarter a year earlier
Concentration	About 60%	Of outbound travel is driven by four specialties: oncology, cardiology, nephrology and orthopaedic surgery
Oncology alone	About 40%	Of cases
Abuja cancer incidence	144.1	Age-standardised rate per 100,000, Abuja Cancer Registry 2009 to 2016
Hypertension, North Central	38.1%	Among the highest regional point estimates in the country
Diabetes, rural Abuja	4.4%	Against a national average of 2.2%

The demand case for the big four is not in doubt. The question is whether this building can serve any of it, and whether anyone has got there first.

02 / WHO ALREADY HAS IT

The Abuja competitive set.

Provider	Position	What they cover
African Medical Centre of Excellence	Quaternary, opened 2025	Oncology, haematology, cardiology. 170 beds to 500, five theatres, three cath labs, 20-bed ICU, 20 chemotherapy chairs, regional stem cell laboratory
Cedarcrest Hospitals	Tertiary private	Cardiac surgery, neurology, orthopaedics, oncology, NICU. The established private reference point in the FCT
Nizamiye Hospital	Tertiary private	General and specialist, the other premier private name in Abuja
NISA Premier Hospital	Private	Maternity, fertility, surgical, diagnostic
Private Care Specialist Fertility	Single specialty	ART, fibroid surgery and obstetrics, Gwarinpa
National Hospital Abuja	Federal tertiary	The public reference centre

Read the first two rows together. Between them AMCE and Cedarcrest hold oncology, cardiac and haematology, with beds, theatres, catheterisation and intensive care behind them. A campus with no beds, no ICU and one lift-less staircase cannot take them on, and should not try.

03 / WHAT THIS BUILDING CAN HOST

Four hard filters.

Filter	What it rules in and out
Ambulatory only	No beds, no intensive care, no overnight stay. Every patient goes home the same day
Sedation on the ground floor only	There is no lift, so a sedated patient cannot be evacuated from the first floor. The theatre and anything needing sedation sit on the ground
One reception	About 98 arrivals a day sustained. A high-volume specialty has to earn its share of that, and an all-day single-patient specialty is worth more per arrival than a high-churn one
Cash-pay	The patients this campus can reach pay out of pocket or through a corporate scheme. Specialties that depend on public reimbursement do not work here

04 / THE SPECIALTIES, ASSESSED

Demand, competition, fit and capital.

Specialty	Demand	Competition	Fit with the building	Capex NGN M	Call
Aesthetics and regenerative	High	Thin	Excellent	25 to 45	BUILD
Hair restoration, FUE	High	Almost none	Excellent	12 to 20	BUILD
Bariatric and metabolic, medical	Very high	Thin	Excellent	8 to 15	BUILD
Longevity and infusion	Growing	Thin	Excellent	15 to 25	BUILD
Dermatology	High	Thin	Excellent	8 to 14	BUILD
Endoscopy and GI day case	High	Moderate	Good, needs the theatre	35 to 60	PHASE 2
Ophthalmology day surgery	Very high	Moderate	Good	45 to 80	PHASE 2
Orthopaedics, day case and paediatric	High	Strong	Good, X-ray is here	10 to 18	PHASE 2
Nephrology and dialysis	Very high	Moderate	Possible, water and space heavy	90 to 160	EVALUATE
Fertility and IVF	High	Strong	Possible	120 to 220	EVALUATE
Oncology	Highest	AMCE, Cedarcrest	No. Needs beds and ICU	very high	AVOID
Cardiac surgery	High	AMCE, Cedarcrest	No. Needs ICU and cath lab	very high	AVOID
Haematology and stem cell	Niche	AMCE	No	very high	AVOID

Capex is the clinical fit-out and equipment for that specialty only. It excludes the building, the theatre and the shared services, which the campus carries anyway. Ranges are indicative and need quotes before anything is committed.

05 / WHAT TO BUILD

Three tiers, and why.

Tier one, now

Five specialties at NGN 68 M to 119 M of clinical capital between them. Every one is cash-pay, ambulatory, uncontested in Abuja at any scale, and already in the plan. This is the tier that makes the campus work, and none of it depends on the theatre.

Tier two, with the theatre

Endoscopy, ophthalmic day surgery and day case orthopaedics all need the theatre or the imaging that is already here, and all three are genuine volume. **Ophthalmology is the one most often overlooked**: cataract surgery is high volume, entirely ambulatory, and the demand in the North is substantial. Day case orthopaedics is worth taking even against Cedarcrest, because fracture follow-up, injections and paediatric work are clinic business that a tertiary centre does not want.

Tier three, evaluate on their own merits

Dialysis and fertility both have real demand and both are ambulatory, but each is a different business rather than another room on this campus. Dialysis needs water treatment, a dedicated plant room and a recurring consumables chain, and the federal subsidy at NGN 12,000 a session has reset what patients expect to pay. Fertility needs a laboratory to a different standard and faces two established Abuja providers. Neither is wrong. Both need their own case.

06 / THE STRATEGIC CONSEQUENCE

What AMCE means for the conversion clinic.

This needs saying before the joint session, not after it. The conversion clinic's thesis is that Nigerian patients needing complex care go abroad, and that we can organise that journey. AMCE's thesis is that those same patients no longer need to. They are competing for one patient, and AMCE is in Abuja with beds, theatres and a King's College Hospital clinical partnership.

That does not kill the conversion clinic. Outbound spend still rose in 2025 and Egypt competes on price and waiting time rather than on proximity. But it does mean two things. The conversion clinic should be positioned on what Egypt does better or cheaper rather than on access as such. And **the campus should not bet on conversion alone**: the ambulatory tier one specialties are the reliable business, and the conversion clinic is the upside on top.

There is a friendlier reading worth testing in the room. A quaternary centre with 170 beds needs ambulatory feeders and does not want clinic work. Lyfe Place could be a referrer into AMCE as readily as a competitor with it, and that conversation is worth having early rather than after both sides have chosen their positions.

07 / WHAT TO VERIFY

Before capital is committed.

Item	Why
Capex quotes	Every figure in section 04 is a range from market reporting, not a quotation. Three quotes per specialty before any order
AMCE's actual service lines	What they have opened with against what they have announced. A 500-bed plan is not 500 beds today, and the gap is where the next two years sit
Abuja cataract and endoscopy volumes	The two tier two specialties where the demand case rests on national rather than FCT data
Dialysis economics after the subsidy	The federal subsidy has moved the price patients expect. A private unit has to be priced against NGN 12,000, not against what it cost last year
Corporate and embassy schemes	Abuja's cash-pay market is unusually weighted to diplomatic and government-linked schemes. Which of them cover which specialty decides the realistic ticket

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Demand figures from Nigerian outbound medical travel reporting for 2024 and 2025, the Abuja Cancer Registry, and published prevalence studies for North Central and North West Nigeria. Competitive detail from provider public statements. Capital ranges are indicative and assembled from market reporting rather than quotations. Not a binding offer.